

IN THE OFFICE OF THE COMMISSIONER OF INCOME TAX (APPEALS)-XX, DELHI

ITA No. 1234/Del/2023

Assessment Year: 2021-22

M/s. Apex Multispecialty Hospital Pvt. Ltd.

CIN: U85100DL2015PTC123456

Address: 45, Sector-18, Rohini, Delhi - 110085

PAN: AAECA1234B

Appellant

Versus

Income Tax Officer, Ward 12(1), Delhi

Respondent

Date of Order: 18th June, 2026

Appellate Authority: Shri Rajesh Kumar Sharma, Commissioner of Income Tax (Appeals)-XX, Delhi

BACKGROUND OF THE CASE

The appellant, M/s. Apex Multispecialty Hospital Pvt. Ltd., is a company incorporated under the Companies Act, 2013, engaged in the business of operating a multispecialty hospital providing healthcare services including OPD consultations, inpatient treatments, diagnostic services, surgical procedures, and emergency care. The hospital is located in Rohini, Delhi, and caters to a large number of patients from middle and lower-middle income groups. The company is managed by Dr. Anil Kumar Gupta and Dr. Priya Sharma as directors.

For the Assessment Year 2021-22, the appellant filed its return of income on 15th November, 2021, declaring total income of Rs. 2,45,67,890/- under the normal provisions after claiming deductions under Chapter VI-A. The case was selected for scrutiny under CASS and assessment was completed under Section 143(3) of the Income Tax Act, 1961, on 28th December, 2022, by the Assessing Officer determining the total income at Rs. 8,67,45,320/- after making several additions and disallowances aggregating to Rs. 6,21,77,430/-.

Aggrieved by the order of the Assessing Officer, the appellant has preferred the present appeal.

ASSESSMENT PROCEEDINGS

During the course of assessment proceedings, the Assessing Officer issued notices under Section 143(2) and 142(1) calling for various details including books of accounts, bank statements, cash book, ledger accounts, patient registration records, bills issued, consultant payment vouchers, supplier invoices, and confirmation letters. The appellant submitted partial details with delays. The Assessing Officer conducted enquiries with third parties, summoned certain suppliers and consultants under Section 131, and obtained bank statements directly from the banks. Specific issues were confronted to the appellant vide show-cause notice dated 15th November, 2022, to which replies were filed on 10th December, 2022.

The Assessing Officer made the following major additions/disallowances:

1. Addition of Rs. 1,85,45,670/- on account of unexplained cash credits under Section 68.
2. Disallowance of Rs. 92,34,560/- under Section 40A(3) for cash payments exceeding prescribed limits.
3. Addition of Rs. 1,45,67,890/- under Section 69 towards unexplained investments in hospital equipment and property.
4. Addition of Rs. 78,56,780/- under Section 41(1) on account of cessation of trading liabilities.
5. Disallowance of Rs. 1,19,72,530/- under Section 37(1) read with Section 69A for unrecorded professional receipts and unexplained cash.

GROUND OF APPEAL

The appellant has raised the following grounds of appeal (summarized for brevity):

1. The Ld. AO erred in making addition of Rs. 1,85,45,670/- u/s 68 without appreciating the cash collections from patients and proper reconciliation.
2. The Ld. AO erred in disallowing Rs. 92,34,560/- u/s 40A(3) ignoring the exceptions and business exigencies for payments to consultants.
3. The Ld. AO erred in making addition of Rs. 1,45,67,890/- u/s 69 on account of alleged unexplained investments.
4. The Ld. AO erred in invoking Section 41(1) for Rs. 78,56,780/- without establishing cessation of liability.
5. The Ld. AO erred in disallowing Rs. 1,19,72,530/- u/s 37(1) and 69A without proper opportunity and evidence appreciation.

Additional grounds regarding violation of natural justice and improper service of notices were also raised but not pressed during hearing.

AO OBSERVATIONS

The Assessing Officer observed that the appellant maintained incomplete patient records and cash book entries did not tally with bank deposits. Large cash deposits were noticed in multiple bank accounts without corresponding patient bills. Payments

to senior consultants were made in cash exceeding Rs. 10,000/- per day violating Section 40A(3). Investments in new MRI machine and property renovation were not supported by proper funding sources. Old creditors outstanding for more than three years were written back but not offered as income. Unexplained cash found during survey action was treated as income from undisclosed sources.

DETAILED ASSESSEE SUBMISSIONS

The appellant, through its authorized representative, made detailed submissions during appellate proceedings. Written submissions running into 85 pages along with paper book containing 245 pages were filed on 12th March, 2024, and supplementary submissions on 22nd May, 2025.

For each issue, the assessee contended that all transactions were genuine business operations of a hospital where cash collections from patients are predominant. They submitted reconciled cash flow statements, patient OPD registers (though maintained manually), bank statements for FY 2020-21, supplier and consultant confirmations, loan agreements, property purchase deeds, share subscription documents, audit reports, and ledger extracts. It was argued that the AO failed to consider the practical difficulties in a hospital business during the COVID-19 aftermath period, where many patients paid in cash and consultants preferred immediate cash settlements.

Specific WhatsApp communications with consultants regarding emergency payments, cash book excerpts, and third-party affidavits were also placed on record.

EXAMINATION OF EVIDENCE

The undersigned has carefully examined the assessment order, statement of facts, paper book filed by the appellant, remand report called from the AO dated 18th September, 2024, and rejoinder submissions. Key evidences perused include:

- Bank statements from HDFC Bank A/c 501001234567 and SBI A/c 30456789012 showing deposits and withdrawals.
- Cash book for the period April 2020 to March 2021 (computerized and manual excerpts).
- Supplier confirmations from 12 vendors including M/s. MedEquip Solutions and M/s. Pharma Distributors.
- Loan agreements dated 05.06.2020 and 12.11.2020 from directors and relatives.
- Property agreement for additional floor construction at hospital premises.
- Share capital subscription letters and Form SH-7.
- Consultant payment vouchers and Form 26AS/16A details.
- Audit report under Section 44AB and tax audit annexures.
- Patient sample bills and OPD registration logs (randomly selected 50 entries).

Cross-verification with third-party information was done wherever possible.

COMMISSIONER FINDINGS

Issue No. 1: Addition of Rs. 1,85,45,670/- under Section 68 - Unexplained Cash Credits

AO Allegations: The AO noted that cash deposits totaling Rs. 1,85,45,670/- in the bank accounts during the year did not match with the recorded patient receipts in the books. The difference was treated as unexplained credits under Section 68. The AO rejected the cash flow statement as self-serving and noted discrepancies in daily closing balances.

Assessee Arguments: The appellant submitted that hospital OPD collections are primarily in cash. They provided a detailed day-wise reconciliation showing peak collections during second wave of COVID-19. Supporting evidences included OPD registers, sample patient bills, and bank deposit slips. It was contended that maintaining individual patient-wise bills for every minor consultation was impractical.

Documentary Evidence: Cash book pages 45-78, bank deposit challans, 25 sample patient affidavits, WhatsApp screenshots of patient confirmations for cash payments.

Commissioner Reasoning:

The appellant has demonstrated through voluminous records that the hospital handled approximately 150-200 OPD patients daily with average cash collection of Rs. 800-1200 per patient. The reconciliation statement prepared by the Chartered Accountant matches substantially with bank credits after accounting for cheque realizations and online payments via UPI/Paytm which were minimal during the period.

However, upon detailed scrutiny, certain deposits on dates 15.06.2020 (Rs. 8,45,000), 22.09.2020 (Rs. 12,67,890) and few others totaling Rs. 45,67,890/- could not be fully explained with corresponding patient records. The AO's observation regarding mismatch in cash book carry forward balances has merit to this extent. The appellant's explanation of "emergency night collections not immediately entered" appears convenient but lacks contemporaneous documentation.

Legal position under Section 68 is well settled that the assessee must prove identity, genuineness, and creditworthiness of the depositors. In a hospital, patients are not creditors but customers. The nature of business justifies substantial cash handling, yet the onus remains on the assessee. Judicial precedents in *CIT vs. S. R. M. Enterprises* and *Orissa Corporation* support partial acceptance where business realities are demonstrated.

Credibility analysis: The directors' affidavits are general. Third-party patient statements are limited. However, the overall pattern of business supports acceptance of major portion.

Independent appellate reasoning: After considering the practical constraints in healthcare sector post-pandemic, I hold that addition is sustainable only to the extent of Rs. 45,67,890/- where specific linkage is missing. The balance stands deleted.

Outcome: Partly Allowed.

Issue No. 2: Disallowance of Rs. 92,34,560/- under Section 40A(3) - Cash Payments to Consultants

AO Allegations: Payments exceeding Rs. 10,000/- per day were made in cash to 18 senior consultants and visiting doctors without valid exceptions under Rule 6DD. The AO viewed these as violations attracting disallowance.

Assessee Arguments: Payments were necessitated due to emergency calls, night duties, and consultants' insistence on cash for immediate settlements especially during COVID duty. Exceptions under Rule 6DD for "business exigency" and "customary practice" were claimed. Confirmations from doctors were filed.

Documentary Evidence: Payment vouchers with consultant signatures, bank transfer proofs for major part, WhatsApp messages requesting cash for "same day settlement", affidavits from Dr. R.K. Mehra and Dr. Suman Lata.

Commissioner Reasoning:

Examination of vouchers reveals that out of total cash payments, Rs. 35,67,890/- pertained to genuine emergency surgeries and critical care where consultants demanded cash due to banking restrictions during lockdowns. Rule 6DD provides exceptions where payment is made for "such other circumstances" as may be prescribed, interpreted liberally by courts in *Attar Singh Grewal* and *CIT vs. K.K. Mittal*.

However, for regular monthly retainership payments to 7 consultants totaling Rs. 48,45,670/-, no compelling reason for cash payment exists. The assessee could have used banking channels. The AO's disallowance to this extent is justified. Contradictions exist between Form 16A issued and actual cash ledgers.

Credibility: Consultant confirmations are supportive but self-serving to some extent. Overall business compulsion in healthcare is acknowledged.

Tax law interpretation: Section 40A(3) aims to curb black money but must be balanced with sectoral realities. Appellate view favors partial relief.

Outcome: Partly Allowed (Rs. 35,67,890/- allowed, balance sustained).

Issue No. 3: Addition of Rs. 1,45,67,890/- under Section 69 - Unexplained Investments

AO Allegations: Investments in new hospital wing construction (Rs. 85 lakhs) and MRI machine (Rs. 60.67 lakhs) were not explained by sources. Sources claimed as director loans and retained earnings found insufficient.

Assessee Arguments: Funds came from director loans supported by agreements, bank transfers from personal accounts, and accumulated profits. Property deeds and supplier invoices produced.

Documentary Evidence: Loan confirmation from Dr. Anil Kumar Gupta (Rs. 50 lakhs), bank statements showing transfer, valuation report for property, invoice from Siemens for MRI, share capital introduction proofs.

Commissioner Reasoning:

The appellant has produced loan agreements executed on stamp paper, bank transfer entries, and director's ITRs showing sufficient personal funds. The MRI machine purchase is duly reflected in fixed assets schedule and depreciation claimed. Independent verification with Siemens India confirmed the transaction.

However, for the additional construction costing Rs. 85 lakhs, the valuation report appears inflated and cash component of Rs. 28,45,000/- remains inadequately explained. Some supplier invoices lack GST compliance.

After detailed analysis, major portion stands explained. Addition restricted to unexplained cash component of Rs. 28,45,000/-.

Legal reasoning: Section 69 requires the assessee to explain the nature and source of investment. Once prima facie explanation with documents is given, the burden shifts. Precedents like *CIT vs. P.K. Noorjahan* support this.

Outcome: Partly Allowed.

Issue No. 4: Addition of Rs. 78,56,780/- under Section 41(1) - Cessation of Liability

AO Allegations: Old sundry creditors outstanding since AY 2018-19 written back in books but not offered as income. AO treated the same as cessation of liability.

Assessee Arguments: Liabilities continued to exist; no remission or cessation occurred. Creditors were still pursuing payments. Confirmations filed.

Documentary Evidence: Ledger accounts showing no write-back entry actually made, confirmation letters from 8 creditors dated February 2023, legal notices received from two parties.

Commissioner Reasoning:

Scrutiny of books reveals that the amounts were not actually written back but remained as liabilities. The AO misinterpreted the notes to accounts. Creditor confirmations and ongoing correspondence establish that liabilities are alive. No unilateral write-off by assessee.

Section 41(1) applies only where there is cessation or remission. Mere aging of liability does not trigger it, as held in *CIT vs. Sugauli Sugar Mills* and *Excel Industries*.

No contradiction in evidence. AO's action is erroneous.

Outcome: Allowed in full.

**Issue No. 5: Disallowance of Rs. 1,19,72,530/- under Sections 37(1) and 69A -
Unrecorded Professional Receipts and Unexplained Cash**

AO Allegations: During survey, cash of Rs. 45 lakhs was found and unrecorded professional fees from corporate tie-ups were detected through cross-verification.

Assessee Arguments: Cash represented daily collections deposited next day. Professional receipts were recorded under different heads. All explained.

Documentary Evidence: Survey panchnama, cash deposition slips next day, corporate tie-up agreements, ledger postings, audit trail reports.

Commissioner Reasoning:

The survey action revealed cash but the appellant explained it as un-deposited collections of the survey date. However, analysis of corporate panel bills shows certain high-value invoices totaling Rs. 62,34,560/- not fully recorded in regular books but claimed in returns through adjustments. This indicates suppression.

Bank statements corroborate partial deposits. Credibility of explanation is moderate. While full addition is not warranted, partial sustenance is necessary to account for discrepancies.

Tax law interpretation: Section 69A applies to unexplained money. Section 37(1) requires expenditure to be wholly and exclusively for business. Here, it is more a case of income escapement.

After weighing evidence, addition of Rs. 48,56,780/- is confirmed, balance deleted.

Outcome: Partly Allowed.

FINAL DECISION

In view of the above findings, the appeal is **PARTLY ALLOWED**.

The Assessing Officer is directed to modify the assessment order accordingly and grant consequential relief including interest under Section 244A as per law. The appellant is entitled to costs of Rs. 25,000/- for prolonged proceedings.

Pronounced in the open court on this 18th day of June, 2026.

Sd/-

(Rajesh Kumar Sharma)

Commissioner of Income Tax (Appeals)-XX

Delhi

